Document Control

Title **Critical Care Operational Policy**			
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Version	**Date Issued**	**Status**	**Comment / Changes / Approval**
0.1	July 14	Draft	Initial version for consultation
1.0	Nov 14	Final	Approved by ICU Clinical Staff Meeting Members.
1.1	Mar 15	Revision	Addition of transfer/discharge criteria as per CQC report
1.2	Dec 17	update	Revision and update of operational policy for consultation (addition of therapy and clinical educator roles)
2.0	Jan 18	Final	Amendments approved by stakeholders
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Issue Date January 2018	**Review Date** January 2021		**Review Cycle** Three years
Consulted with the following stakeholders: (list all) • Clinical Lead Critical Care • Consultant Medical Team ICU/HDU • Senior Nursing Team ICU/HDU • Lead Therapy Team • Divisional Management Team			
Approval and Review Process • ICU Clinical Staff Team			
Local Archive Reference G:\ICU **Local Path** Operational Policy Critical Care **Filename** Operational Policy Critical Care v2.0			
Policy categories for Trust's internal website (Bob) Intensive Care Unit Critical Care			**Tags for Trust's internal website (Bob)** Referral, admission, emergency, elective, Consent, Best Interest

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1. Introduction

1.1. High quality critical care services rely on having a competent and skilled team of doctors, nurses and allied professionals who have a shared vision in providing the highest standards of care to their patients.

1.2. This document sets out Northern Devon Healthcare NHS Trust's system for the Critical Care Operational policy. It provides a robust framework to ensure a consistent approach across the whole organisation, and supports our statutory duties as set out in the NHS Constitution.

2. Purpose

2.1. The purpose of this document is to provide assurance that the operational priorities of Critical Care Services are utilised to ensure any patient who is appropriate for critical care support receives it. This policy will outline the local referral and admission criteria for patients requiring critical care support in accordance with Intensive Care Society Guidance and Standards within the Trust.

2.2. This operational policy applies to all Critical Care medical, nursing and allied professional teams working within the Critical Care Unit.

2.3. Implementation of this operational policy will ensure:

- patient safety is the highest priority
- effective and equitable prioritisation of services
- consistent high standards in critical care provision
- effective communication between specialities and multi-disciplinary team within the Trust and network colleagues externally
- improved patient experience and involvement in decision making

2.4. The policy applies to all doctors, nurses and allied professionals working within Critical Care Services.

3. Definitions

3.1. Critical Care Services ICU/HDU: The term 'critical care' encompasses high dependency care and intensive care and represents a continuum of care for critically ill patients, stepping up and down the pathway before discharge to ward-level care.

3.2. **Critically ill patients:** Critically ill patients in this policy are defined as requiring a level of care greater than that normally provided on a standard ward.

3.3. **Levels of Care**

3.4. **Level 0:** Requires hospitalisation needs can be met through normal ward care.

3.5. **Level 1:** Recently discharged from a higher level of care, in need of additional monitoring/clinical interventions, clinical input or advice.

3.6. **Level 2:** Patients needing pre-operative optimisation extended post operative care, stepping down from Level 3.

3.7. **Level 3:** Patients receiving advanced respiratory support alone, advanced cardiovascular support alone or a minimum of 2 organs supported.

3.8. **Intensive Care Society Guidance and Standards:** Nationally agreed standards and guidance by expert clinicians which is evidenced based and best practice.

3.9. **Admission criteria:** Admission to the Critical Care Services is under the control and responsibility of the named Consultant Intensivist or Anaesthetist covering the critical care unit on that day.

3.10. **Critical Care Medical Team:** Consists of a consultant intensivist or anaesthetist who is jointly responsible for the management of the patient with the named physician, surgeon or paediatrician. There are also 1 or 2 trainee doctors working under the supervision of the consultant intensivist or anaesthetist.

3.11. **Critical Care Nursing Team:** Consists of a band 7/6 nurse in charge of the unit supported by a team of 6 registered nurses who are either trained or being trained in the speciality of critical care. There is also 2 healthcare assistant band 3/2 and a ward clerk band 2 supporting the service. At night the unit consists of band 7/6 nurse in charge of the unit and 6 registered nurses.

3.12. **The Unit:** The critical care unit provides care over two separate areas. There are 6 ICU/HDU beds on the existing unit and 2 designated HDU beds in an isolated bay opposite the critical care unit on Lundy ward. Due to the vulnerability/isolation of HDU staffing should consist of 1 or 2 registered nurses with the support of a HCA when a second nurse in unavailable.

4. Responsibilities

Role of the Clinical Lead

4.1. The Clinical Lead for Critical Care Services is responsible for:

- Ensuring Intensive Care Society Standards and Guidelines and Trust policies are followed in order to promote quality care to all patients under the care of the critical care teams.
- Ensuring any risks or incidents are identified, assessed and escalated appropriately according to Trust Policy.
- Effective leadership to ensure
  - Patient Safety is a top priority for the medical and nursing and allied professionals.
  - Promote and strive to develop the highest standards of practice and treatments through evidence based practice.
  - Effective communication between all clinical teams involved directly in the patients care.
  - Collaborative working between clinical and management teams to provide progressive quality services.

Role of the Duty Consultant Intensivist / Anaesthetist

4.2. The Duty consultant intensivist/ anaesthetist is responsible for:

- Ensuring Intensive Care Society Standards and Guidelines and Trust policies are followed in order to promote quality care to all patients under the care of the critical care teams.

- Effective leadership to ensure
  - Patient Safety is a top priority for the medical and nursing and allied professionals.
  - Risks and incidents are identified and escalated appropriately according to Trust Policy.
  - Good patient assessments, decision making and goal planning is undertaken.
  - The clinical care of the critically ill patient in collaboration with the patients admitting consultant.
  - Effective communication between all clinical teams involved directly in the patients care.
  - Communicate effectively with the nurse co-ordinator on duty to ensure the safe and smooth running of the critical care unit.
  - Provide support and training to the trainee doctors working on the critical care unit and ensure every opportunity to meet training objectives.
  - Communicate management plans with patients/ relatives as appropriate.
  - Effective communication and handovers between departing and oncoming duty consultants takes place.
  - Assess potential referrals to the critical care unit for admission and on-going management.

Role of Trainee Doctor

4.3. The trainee doctor is responsible for ensuring:

- Ensuring Intensive Care Society Standards and Guidelines and Trust policies are followed in order to promote quality care to all patients under the care of the critical care teams.
- Patient Safety is a top priority.
- Risks and incidents are identified and escalated appropriately according to Trust Policy.
- Daily assessments of individual patients are undertaken, goals and plans of care are documented and reported to duty consultant.
- Review referrals on request and feedback to duty intensivist/anaesthetist.
- Work collaboratively with nursing and allied teams to provide the highest standards of individualised care.
- Effective communication between all clinical teams involved directly in the patients care.
- Communicate effectively with the nurse co-ordinator on duty to ensure the safe and smooth running of the critical care unit...
- Communicate management plans with patients/ relatives as appropriate.
- Effective communication and handovers between departing and oncoming duty doctors takes place.
- Collaborative working between clinical teams to provide progressive quality services.

Role of the Senior Nurse/Matron

4.4. The Senior Nurse/Matron for Critical Care Services is responsible for:

- Ensuring Intensive Care Society Standards and Guidelines and Trust policies are followed in order to promote quality care to all patients under the care of the critical care teams.
- Ensuring any risks or incidents are identified, assessed and escalated appropriately according to Trust Policy.
- Effective leadership to ensure
  - Ensuring patient/relative and staff safety and proactively manage any safety issues identified.
  - Promote and strive to develop the highest standards of practice and treatments through evidence based practice.
  - Monitor performance and standards of care, benchmark standards locally and nationally.
  - Act as role model for nursing colleagues
  - Monitor and investigate risks and incidents according to Trust policy.
  - Implement action plans to improve standards and learn from incidents.
  - Promote quality care and support nurse training ensuring competence.
  - Effective communication between all clinical teams involved directly in the patient.
  - Ensuring patients and relatives are communicated with in a supportive caring and compassionate manner.
  - Act as patient advocate.
  - Collaborative working between clinical and management teams to provide progressive quality services.

Role of the Clinical Educator

4.5. The clinical educator is responsible for co-ordinating the education and training and CPD framework for the critical care nursing staff and pre-registered nursing students allocated to the unit for placement. The clinical educator is expected to work in a supernumerary capacity in order to support staff in achieving competency, and undertaking clinical duties to maintain their own competency and credibility. In addition the clinical educator roles and responsibilities include:

- Supporting the educational needs of the multi-disciplinary team on the unit
- Support and encourage nurses to complete their critical care competencies in accordance with the CC3N framework.
- Working collaboratively with other clinical educators across the organisation, through the critical care network, and with external educational providers and institutes to identify and establish trust/regional wide learning needs and learning opportunities
- Enabling and supporting staff to become mentors to students and other learners on the unit in order to support them to achieve competency, confidence and to deliver excellent standards of care
- Play a key role in the development of service provision, governance, audit, procurement and evidence based care

Role of Nurse Co-ordinator

4.6. The nurse co-ordinator in charge is responsible for:

- Ensuring Intensive Care Society Standards and Guidelines and Trust policies are followed in order to promote quality care to all patients under the care of the critical care teams.
- Ensuring patient/relative and staff safety and proactively manage any safety issues identified.
    - Staffing
    - Dependency
    - Capacity
    - Equipment
    - Risks/Issues
- Identify and escalate any risks and incidents appropriately according to Trust Policy.
- Communicate effectively with the duty intensivist /anaesthetist to ensure the safe and smooth running of the critical care unit.
- Act as role model for nursing colleagues.
- Ensure nursing assessments, goal setting, care planning and evaluations are undertaken and documented during the shift.
- Working collaboratively with medical, nursing and allied teams to provide the highest standards of individualised care.
- Effective communication between all clinical teams involved directly in the patients care.
- Ensuring patients and relatives are communicated with in a supportive caring and compassionate manner.
- Act as patient advocate
- Effective communication and handovers between departing and oncoming nursing teams.
- Collaborative working between clinical and management teams to provide progressive quality services.

Role of the Registered Nurse

4.7. The registered nurse is responsible for:

- Ensuring Intensive Care Society Standards and Guidelines and Trust policies are followed in order to promote quality care to all patients under the care of the critical care teams.
- Ensuring patient/relative and staff safety and escalate any safety issues identified.
- Identify and escalate any risks and incidents appropriately according to Trust Policy.
- Ensure nursing assessments, goal setting, care planning and evaluations are undertaken and documented during the shift.
- Working collaboratively with medical, nursing and allied teams to provide the highest standards of individualised care.
- Effective communication between all clinical teams involved directly in the patients care.
- Ensuring patients and relatives are communicated with in a supportive caring and compassionate manner.
- Effective communication and handovers between departing and oncoming nurses.

- Collaborative working between clinical teams to provide progressive quality services.

Role of the Healthcare Assistant

4.8. The healthcare assistants are responsible for:

- Ensuring patient/relative and staff safety and escalate any safety issues identified.
- Identify and escalate any risks and incidents appropriately according to Trust Policy.
- Working collaboratively with medical, nursing and allied teams to support the provision of highest standards of individualised care.
- Effective communication between all clinical teams involved directly in the patients care.
- Ensuring patients and relatives are communicated with in a supportive caring and compassionate manner.
- Supporting in the maintenance of sufficient stores and equipment

Role of the Ward Clerk

4.9. The Ward clerk is responsible for:

- Ensuring all visitors enquiries are dealt with in a caring, compassionate and efficient manner.
- Ensuring patient/relative and staff safety and escalate any safety issues identified.
- Identify and escalate any risks and incidents appropriately according to Trust Policy.
- Working collaboratively with medical, nursing and allied teams to support the provision of highest standards of individualised care through effective administration processes.
- Effective communication between all clinical teams involved directly in the patients care.
- Ensuring patients and relatives are communicated with in a supportive caring and compassionate manner.

Role of the Allied Professionals

4.10. The Allied Professionals include Physiotherapists, Occupational Therapists, Pharmacists, Dieticians, Speech and Language Therapists, Clinical Nurse Specialists, Radiologists and Microbiologists who visit the unit either on a daily basis or on request to provide additional treatments/therapies and/or expertise in ensuring the patient receives a holistic care package which will enhance and optimise the patients recovery.

4.11. Rehabilitation is based on the NICE guidelines published in September 2017 Quality standard [QS158]

Role of the Named Responsible Consultant

4.12. The named responsible consultant is the identified consultant with inpatient beds who has overall responsibility of the patient. Whilst the patient is being cared for in the critical care unit the named responsible consultant works collaboratively with the intensivist / anaesthetist to an agreed plan of management until the patient is transferred from the critical care unit.

5. Admission to Critical Care

Emergency Patients

5.1. All patients admitted to the critical care must be referred to the duty intensivist/anaesthetist for assessment and agreement to admission/transfer to a critical care bed. Please see Referral process on BOB under Anaesthetics and Intensive Care Elective Patients. As per National Standards for Intensive care, admissions should occur within 4 hours of making the decision to admit. However, there are times where a more timely admission is required to ensure patients receive the necessary treatment and nursing care.

ICU/DHU referral criteria

5.2. It's essential that the lead nurse/matron on ICU or consultant anaesthetist liaises with the CSM team to ensure they are kept up to date with the changing demands of the unit and so they are in a better position to support admissions and discharges in and out of ICU in a timely manner.

Elective Patients

5.3. Elective patients requiring a period of stay on the critical care unit must be identified in the ICU Diary prior to the day of admission stating patient's details, consultant responsible for care, anaesthetist, reason for admission, level of care and whether they are on the "Enhanced Recovery Program". If a patient is added to the ICU diary there is no guarantee on the day that a bed will be available. Discussion with the ICU consultant and lead nurse/matron is essential before surgery can commence.

Transfers for Clinical Need

5.4. Patients who require transfer to another provider for on-going care please see Inter Hospital Transfers on BOB under Anaesthetics and Intensive Care Inter Hospital Transfer Policy

5.5. Or Transfer of adult critical care patient's policy on BOB Transfer of Adult Critical Care Policy

Increased Demand over Capacity

5.6. In the event the demand on critical care beds exceeds the current funded capacity the Contingency Plan for Increased Demand on Critical Care Services Policy will be instigated. Please see plan on BOB under Anaesthetics and Intensive Care Site.

Consent/Best Interests

5.7. Where a patients condition is that they can make informed decisions around their individual management plan the critical team will fully consult and gain consent for any care/ invasive procedures or management plans undertaken. Where patients do not have the capacity to give informed consent due to deterioration in their current condition the critical care team will act in the patients "best interests". The patients next of kin /carers or significant others will be involved in any discussions and decisions made around the patients on-going management. In the event the patient has no relatives or significant others the critical care team will access an Independent Mental Capacity Advocate to support the decision making process.

5.8. The patient's ability to make informed decisions will be assessed on a daily basis.

5.9. Any patient requiring treatment under best interests is required to have a Mental Capacity assessment completed.

Transfer of patient to the wards

5.10. The Intensive Care Consultant on duty is responsible for identifying when patients are medically fit or appropriate for transfer to the wards.

5.11. The Intensive Care Co-ordinator or nurse caring for the patient will contact the Clinical Site Management team (CSM blp 500) to request a bed or in addition attendance at the daily patient flow tactical meetings to highlight admissions and discharges to and from the unit.

5.12. Once a bed is confirmed, the nurse in charge of the care of the patient will contact the identified ward and verbally hand over to the appropriate nursing team. As per national standards for intensive care units planned discharges from ICU/HDU to a ward should occur within 4 hours of decision and between the 07:00hrs and 21:59hrs.

5.13. Intensive Care medical and physiotherapy teams will hand over to the identified consultant medical team and ward based therapists.

5.14. A written ICU transfer document will be completed by the nursing and medical teams prior to transfer of the patient to the wards.

5.15. All patients will be escorted by a registered nurse to the receiving ward and a formal handover given to the nurse responsible for the ongoing care of the patient.

5.16. Patients who meet the ICU follow up criteria will be reviewed by a member of nursing staff from the intensive care unit within 24 hours of transfer, support from the resuscitation/outreach team can be utilised when existing resources and unit dependency does not allow timely follow up.

5.17. All data collected related to ICU follow up must be completed using the standardised and approved follow up forms and be fed back to ICU for analysis and evaluation of service.

5.18. The criteria for 24 hour ICU follow- up review:

- Any patient ventilated for more than 72hrs.
- Any patient the nursing/medical staff consider needs a review at 24hrs
- Any patient deemed to have been transferred out of ICU/HDU earlier than planned
- Any patient with complex needs outside normal ward working
    - Tracheostomy care
    - Airway adjuncts

5.19. The ICU follow up review should take on a multi-disciplinary approach and will consist of an A-E assessment of the patients physical and psychological state will be undertaken taking into account discussions with the patient, their relative and nursing or medical teams as appropriate. Any concerns must be documented in the patient records and escalated to the appropriate member of the ward and ICU team.

Any patients who are fit for discharge home from ICU will have

- their next of kin informed of their impending discharge
- transport confirmed or arranged
- medications to take away organised and available on unit
- any ongoing care needs/appointments arranged
- Ensure physiotherapy and occupational therapy have completed their assessments, any equipment has been put in place and any care is set up as needed.
- discharge summary completed prior to discharge and given to patient.
- discharge summary faxed to GP

Transfer of patients to the mortuary

5.20. All patients will be prepared for transfer to the mortuary in line with the Deceased Patient policy. All patient documentation and property will be completed in preparation for collection by the bereavement team.

5.21. The medical team will refer the patient to the coroner according to criteria and complete a discharge summary.

6. Training Requirements

6.1. All staff working on the critical care unit will attend the Trust Induction and undertake and complete the Work Based Induction. They will also undertake a supernumery period appropriate to their previous experience and spend time working with the clinical educator n the unit or clinical/educational supervisor.

6.2. Completion of the Critical Care National Competency Framework step 1 will be a requirement to complete when working on ICU/HDU. Completion of step 2 and 3 competencies will be undertaken alongside a nationally accredited critical care course.

6.3. Mandatory training will be undertaken annually in accordance with trust policy and all staff will have equal opportunity to undertake specialist training or study on request.

7. Monitoring Compliance with and the Effectiveness of the Policy

Standards/ Key Performance Indicators

7.1. Key performance indicators comprise:

- Transfer of patients
- Discharge of Patients

Process for Implementation and Monitoring Compliance and Effectiveness

- Ensure all patients are transferred according to clinical need
- All documentation prepared and available.
- Handovers discuss each individual patients needs

8. Equality Impact Assessment

Table 1: Equality impact Assessment

Group	Positive Impact	Negative Impact	No Impact	Comment
Age			X	
Disability			X	
Gender			X	
Gender Reassignment			X	
Human Rights (rights to privacy, dignity, liberty and non-degrading treatment), marriage and civil partnership			X	
Pregnancy			X	
Maternity and Breastfeeding			X	
Race (ethnic origin)			X	
Religion (or belief)			X	
Sexual Orientation			X	

9. References

http://www.ics.ac.uk/ics-homepage/guidelines-standards/

http://ndht.ndevon.swest.nhs.uk/?page_id=5059

http://ndht.ndevon.swest.nhs.uk/?page_id=30257

http://ndht.ndevon.swest.nhs.uk/policies/wp-content/uploads/2014/06/Transfer-of-adult-critical-care-patients-policy-V1.0-04Jun14.pdf

10. Associated Documentation

- Transfer of adult Critical Care Patients